

30 providers · 5 months on Abridge · 60% utilization · 24,000 documented encounters

AT A GLANCE

Your 30 providers are generating \$3K each annually in documented value. At 60% utilization, you're capturing \$79K today—with a clear path to \$342K at scale.

\$79K

ANNUAL VALUE CREATED
Defensible hard dollars

\$3K

VALUE / PROVIDER
Based on 30 providers

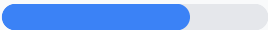
+\$262K

EXPANSION POTENTIAL
At 90 providers

DEPLOYMENT MATURITY

Utilization Rate

60%

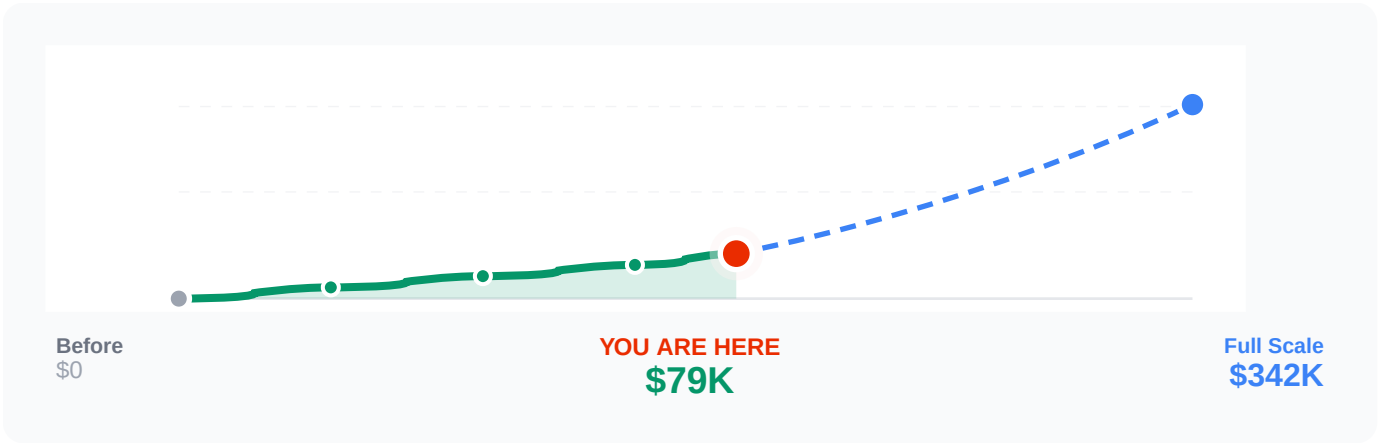


Developing

VALUE DRIVERS

wRVU Per Encounter

YOUR VALUE JOURNEY



VALUE BREAKDOWN



Core Financial Value

Directly measurable revenue impact

\$79K

WRVU Per Encounter 24,000 encounters x \$33/wRVU x 50% attribution

\$79K



Operational Efficiency

Time and workflow improvements

Time Saved

2,800 hours/year

Chart Closure Improvement

+12%

KEY TAKEAWAY The foundation is solid. Your deployment is proving out the value model in your environment—the next step is deepening adoption among existing providers and extending to new ones.

● WRVU PER ENCOUNTER

Revenue capture improvement

\$79K
annual value

BEFORE ABRIDGE

2.30

wRVU/enc



AFTER ABRIDGE

2.50

wRVU/enc

=

+0.20

9% lift

BENCHMARK COMPARISON

Typical range: 3-7% lift

● Above typical range

VALIDATION RECOMMENDED

Your results exceed our typical range. This isn't necessarily wrong—some organizations do see higher lift, especially if baseline documentation was sparse. But we recommend confirming: Was baseline measured the same way as current? Were there other coding or documentation initiatives during this period? Is the provider population consistent between periods?

VALUE CALCULATION

+0.20 wRVU/enc × 24,000 encounters × \$33/wRVU × 50% attribution = \$79K

We use Medicare's \$33 conversion factor and 50% attribution to Abridge. If your payer mix is commercial-heavy (where conversion factors run \$45-65), actual revenue impact may be 30-50% higher.

WHAT THIS MEANS

Here's what we're seeing: A 8.7% lift in wRVU typically means your documentation is now capturing clinical complexity that was always there—providers aren't doing more, the notes are just reflecting reality better. Your 9% lift is above our typical 3-7% range, which is interesting. This could mean several things: your baseline documentation may have been particularly sparse, or you have complex patient populations that benefit more from thorough capture. It's worth exploring—if you can validate the baseline methodology, this becomes a powerful proof point.

TIME IN NOTES

Documentation efficiency

7.0 min/encounter
saved per encounter

BEFORE ABRIDGE

10.00

min/encounter



AFTER ABRIDGE

3.00

min/encounter

=

+7.00

70% reduction

TREND OVER 6 MONTHS

IMPROVING



BENCHMARK COMPARISON

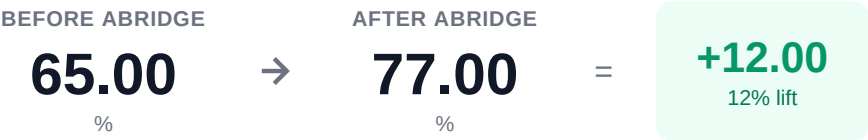
Typical range: 3-5 min reduction

Above typical

WHAT THIS MEANS

Let's think about what 7 minutes saved really means: That's 2,800 hours annually—time that was previously locked up in documentation. The interesting question is: where does that time actually go? We've seen organizations channel it three ways: back to patients (more access, longer visits), back to providers (better work-life balance), or to operations (overtime reduction). Which path makes sense for you depends on your current priorities. The 7-minute reduction you're seeing is meaningful—it's the kind of lift that providers notice in their daily rhythm.

SAME-DAY CHART CLOSURE
Documentation timeliness



BENCHMARK COMPARISON
Typical range: 10-20 percentage point improvement

● Within typical range

WHAT THIS MEANS
Chart closure is one of those metrics that tells a bigger story. When charts close same-day, it usually means providers are documenting in real-time rather than batching at the end of the day (or week). Your 12 percentage point improvement suggests the workflow is clicking—providers are trusting the AI draft enough to finalize notes between patients. Beyond the workflow signal, there's a downstream revenue impact: faster closure means faster billing cycles and reduced compliance exposure. Worth watching as your deployment matures.

💡 EARLY INDICATORS

🕒 WHAT'S WORKING WELL

- Revenue capture is responding—9% wRVU lift suggests documentation is better reflecting clinical complexity
- Providers are reclaiming time—7 min/encounter freed up, which compounds across your volume
- Real-time documentation emerging—12pt closure improvement suggests workflow is clicking

🕒 AREAS TO WATCH

- Utilization at 60%—worth exploring what's keeping remaining providers from adopting. Training gaps? Specialty-specific barriers?
- wRVU lift above typical—exciting if validated. Consider auditing baseline methodology to confirm this is real lift vs. measurement change
- Time savings currently unrealized—the hours are there, but haven't yet converted to patient access or reduced overtime. Worth a conversation with ops.

↗️ OPTIMIZATION OPPORTUNITIES

1 DEEPEN ADOPTION

+\$23K/yr

CURRENT

60% of providers using consistently



TARGET

80-85% (where value compounds)

RECOMMENDED ACTION

Look for patterns: Which specialties or sites are lagging? Often there are 2-3 common barriers that, once addressed, unlock the next wave of adoption.

2 CONVERT TIME TO VALUE

+\$63K/yr

CURRENT

2,800 hrs/year reclaimed but not monetized



TARGET

15-20% converting to patient access or OT reduction

RECOMMENDED ACTION

This is a collaboration opportunity with scheduling/ops. The time savings are real—the question is how to redirect that capacity intentionally rather than letting it dissipate.

3 VALIDATE YOUR STANDOUT RESULTS

+\$0/yr

CURRENT

9% wRVU lift (above typical)



TARGET

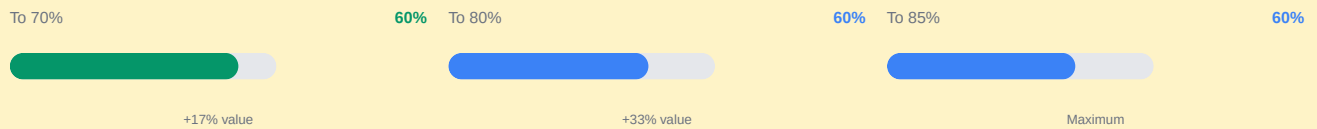
Confirmed, defensible methodology

RECOMMENDED ACTION

High-lift results are great news if they hold up to scrutiny. Quick audit: Was baseline measured consistently? Any other coding initiatives running in parallel? If you can answer "yes" to both, this becomes a compelling proof point.

🔍 EARLY STAGE CONTEXT

At 60% utilization and 5 months in, you're still in early deployment. Here's the path forward:



📈 THE BIGGER PICTURE

Here's what's worth noting: at 60% utilization, you're seeing \$79K annually with significant runway remaining. The value you're capturing today is real—and it's just a fraction of what's possible as adoption deepens.

KEY TAKEAWAY We've identified 3 optimization areas worth exploring. Start with whichever feels most actionable—even partial progress on any of these moves the needle meaningfully.

THE PATH TO FULL SCALE

THE EXPANSION STORY

You've established proof of concept. The question now is: how does this scale? The math suggests each additional provider at target utilization adds roughly \$4K annually.

\$342K

PROJECTED ANNUAL VALUE
At full scale

+\$262K

ADDITIONAL VALUE
Beyond current

90

TARGET PROVIDERS
75% utilization

HOW WE CALCULATE YOUR EXPANSION

Current annual value	\$79K
Provider scale	3.0x
Utilization improvement	1.25x
Projected annual value at scale	\$342K

CURRENT VS. FULL SCALE

CURRENT STATE

Providers:	30
Utilization:	60%
Annual value:	\$79K
Value/provider:	\$3K



FULL SCALE TARGET

Providers:	90
Utilization:	75%
Annual value:	\$342K
Added value:	+\$262K

Think of your current deployment as a pilot that's already delivering. The patterns you're seeing at 30 providers translate predictably as you scale—this isn't a leap of faith, it's an extension of demonstrated results.

KEY TAKEAWAY The model works in your environment. The expansion opportunity is about replicating what's already proven—methodically extending to new providers while deepening adoption with existing ones.

HOW WE CALCULATED YOUR VALUE

Every calculation here traces back to either your data or industry-standard benchmarks. We lean conservative throughout—no optimistic assumptions. The methodology is transparent so you can validate the logic yourself.

YOUR INPUTS

- 30 providers
- 40,000 annual encounters
- 60% utilization rate
- 5 months on Abridge
- 24,000 documented encounters

VALUE ATTRIBUTION

- wRVU: \$33/wRVU (Medicare), 50% attribution
- Time: \$150/hr provider value
- Retention: \$500K per departure
- Commercial payers: 30-50% higher

BENCHMARK RANGES

TYPICAL ABRIDGE RESULTS

- wRVU lift: 3-7%
- Time reduction: 3-5 min/enc
- Chart closure: +5-15 pp

QUALITY OF LIFE

- Pajama time: -2-5 hrs/wk
- Satisfaction: +10-20 pts
- Burnout: varies by org

WHAT WE DON'T INCLUDE

- Downstream revenue from patient experience
- Quality measure incentives (MIPS, HEDIS)
- Reduced compliance/audit risk
- Training time reduction & career satisfaction

Have questions about any calculation? We're happy to walk through the details—every number here is designed to be defensible in a CFO conversation.

KEY TAKEAWAY Conservative by design: with commercial payer rates and full attribution, actual value is typically 30-50% higher than shown here.